The Society of Diagnostic Medical Sonography (SDMS) appreciates the opportunity to comment on this Request for Information regarding the adoption and use of artificial intelligence in clinical care. As the professional organization representing diagnostic medical sonographers, the SDMS supports thoughtful integration of AI technologies that enhance diagnostic quality, support clinical decision-making, and improve patient safety. Note: AI tools should complement, not replace, the expertise of credentialed sonographers and should not be used to justify reduced staffing, compressed exam times, or expanded productivity expectations.

As AI tools are increasingly incorporated into imaging and clinical workflows, it will be important that their use complements, rather than complicates, frontline clinical practice and is implemented in a manner that supports appropriate clinical judgment and accountability. Policies should promote transparency, appropriate clinical context, and safeguards that prevent unintended increases in workload or erosion of diagnostic quality. The SDMS encourages continued engagement with clinical professionals who use AI-enabled tools in real-world care settings and stands ready to serve as a resource as this work progresses.

Additional Questions/Answers

1. **What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?**

   In sonography, barriers include limited integration of AI tools into real-time imaging acquisition workflows, inconsistent interoperability with PACS and EHR systems, and insufficient validation of AI performance in diverse clinical environments. Tools developed without early and meaningful input from sonographers may not align with clinical realities, reducing usability and slowing adoption.

2. **What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?**

   HHS should prioritize clarity around the use of AI as a clinical support tool, rather than as a substitute for clinical judgment. Payment and programmatic approaches should encourage adoption of AI tools that demonstrate improved quality or efficiency without introducing additional documentation requirements or productivity pressures that could undermine diagnostic integrity.

3. **For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and**

implementation issues exist and what role, if any, should HHS play to help address them?

AI use in imaging raises questions related to accountability, liability, transparency, and data governance. HHS can play a constructive role by supporting policies that reinforce clinician oversight, define expectations for transparency in AI outputs, and promote clear accountability structures that protect patients and clinicians alike.

4. **For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?**

   Effective evaluation methods should assess not only technical accuracy but also clinical usability, workflow impact, and effects on diagnostic quality. The SDMS encourages HHS to support evaluation frameworks that include real-world clinical testing across varied practice settings and patient populations.

5. **How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?**

   HHS could support collaboration between technology developers, professional organizations, and third-party evaluators to establish best practice frameworks for clinical AI use. Inclusion of frontline clinical perspectives in evaluation and certification processes would help ensure AI tools are both safe and practical.

6. **Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?**

   AI has shown promise in areas such as automated measurements and workflow support but has fallen short when tools are deployed without sufficient clinical context or when they increase the validation burden on clinicians. Continued emphasis should be placed on tools that demonstrably reduce redundant tasks while maintaining or improving diagnostic quality.

7. **Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?**

AI adoption is often driven by organizational leadership and informatics teams, while successful implementation depends on acceptance by frontline clinicians. Administrative hurdles include lack of standardized implementation guidance, limited training resources, and unclear success metrics.

8. **Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care?**

   Enhanced interoperability of imaging data, including ultrasound images and reports, would expand opportunities for AI development and validation. Standardized data access would help ensure AI tools are trained and evaluated using clinically representative datasets.

9. **What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?**

   Patients and caregivers are interested in AI that improves diagnostic accuracy, reduces unnecessary repeat imaging, and supports timely care, while maintaining clinician involvement and accountability. Concerns include data privacy and the potential for AI to be used in ways that prioritize efficiency over individualized care.

10. **Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?**

    a. **Are there published findings about the impact of adopted AI tools and their use clinical care?**

    b. **How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?**

    The SDMS encourages HHS to prioritize research on AI applications that support imaging acquisition quality, workflow efficiency, and integration of clinical context. Research should also examine the impact of AI on workload, patient outcomes, and equity across diverse populations and care settings.